

LITTLE BLUE BOOK NO. 1242
Edited by E. Haldeman-Julius

**The Care of the
Skin and Hair**
And Other General Health Hints
Morris Fishbein, M. D.

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*** START OF THE PROJECT GUTENBERG EBOOK THE CARE OF
THE SKIN AND HAIR ***

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The Care of the Skin and Hair

And Other General Health Hints

Morris Fishbein, M. D.

**Editor Journal American Medical Association
and Hygeia, the Health Magazine.**

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CARE OF THE SKIN AND HAIR AND OTHER GENERAL HEALTH HINTS

SKIN DISEASES AND THEIR CURE

There are still persons who believe that every disease of the skin can be successfully treated with a salve, a lotion or a powder. This belief and the advertisements of nostrums for the treatment of skin disease are a reflection of the actual knowledge of such conditions held even by the medical profession a quarter of a century ago.

Today the methods of treatment of skin diseases include practically every form of apparatus, every method of medical administration known to medical science. This advance is a reflection of the application of knowledge in physics, chemistry, biology and bacteriology to medicine, and of a more thorough comprehension of the fact that the skin is not merely a protective covering for the body but an organ whose condition reflects that of the other tissues and influences them definitely.

Perhaps the greatest advance has been the X-ray and the knowledge that application of its rays would affect not only tumors, whose cause is unknown, but also inflammations and degenerations of the skin produced by parasitic organisms. Not only the rays of the X-ray tube, but also those of radium, of the sun and of the ultraviolet lamps are known to have definite effects on the skin. Radium is used for destroying cancer of the skin, masses of veins, birth marks, moles and similar unsightly excrescences. As Dr. Fred Wise indicated at a recent meeting of specialists in diseases of the skin, the physician is no longer limited to any single form of treatment, but may attack such growths with any of the destructive rays that have been mentioned; he may freeze them with carbon dioxide snow; he may remove them surgically with the knife; he may dry them by passing an electric current through them, or cut them with a needle whose cutting power depends on vibrations produced by electric waves. In attacking certain parasitic diseases of the skin, the specialist reaches them through the blood, injecting combinations of dyes and metallic elements or specific drugs which produce the immediate death of the parasitic organisms when they come in contact with them.

There still remain infections and diseases of the skin which are not easily amenable to treatment. Pemphigus, in which there are tremendous wheals, blisters and similar eruptions, is not known to yield easily to any form of treatment, although it may be benefited on occasion by the use of drugs such as arsenic and quinine. Generalized changes of the nature of the skin resulting from disturbances of the nervous system and of the glands sometimes resist every form of medical attack. Research in the field of skin diseases is one of the most promising opportunities for investigation to the medical man with research instincts.

COSMETICS

Since investigation has been undertaken by physicians and chemists there is less and less likelihood of the sale or purchase of cosmetic preparations, including face creams, powders and lotions, containing poisonous ingredients, although at one time such metallic poisons as lead, bismuth, or arsenic could be found as ingredients of such beautifiers. Nowadays, the difficulty seems to be that there are a multiplicity of preparations which have no real warrant, since they differ only in unessential qualities from each other. There are, for example, dozens of creams sold for application to the skin with claims that they nourish the skin, when, as a matter of fact, there is no such thing as a skin food. All of these creams essentially are cold creams, modified by varying amounts of perfumes or other thinning or thickening factors.

These statements do not apply, however, to the creams that are used as depilatories. Most of the widely advertised ointments for this purpose have as their basis a salt of the metal barium. Application of the ointment removes the hair, but does not affect the growing end within the skin. In most instances the new growth of hair is thicker and coarser and darker than the original growth removed by the depilatory. In some instances, superfluous hair is removed by the application of the X-ray. It has been found, however, that X-ray machines seldom kill the living cell from which the hair grows, unless it is applied in dosage so strong as to produce definite changes in the skin itself.

Specialists in diseases of the skin are being called on to treat thickening of the skin, or so-called precancerous keratosis, resulting from the use of the X-ray for the removal of hair by inexperienced persons. Those who give special attention to these problems are agreed that the one certain method of removal of superfluous hair is the use of the electric needle. It must be understood, however, that it is possible to remove only a few hairs at a single treatment, and that the method requires great expertness in its application.

SKIN DISEASE QUACKERY

Faith springs eternal in the human breast and credulity is not the property of any single class. One finds our best educated citizens just as easily susceptible to false and fraudulent claims as the most ignorant. Indeed, in many instances, particularly in relation to medical science, a little knowledge is a dangerous thing, since its possessor is likely to be impressed with his own ability to make suitable judgments.

Of particular interest in this connection are the many cosmetics offered to women who seek eternally for artificial beauty. According to an investigation recently made, the American public paid, in 1921, \$150,000,000 for perfumery and cosmetics. There were purchased 240,000,000 packages of talcum and face powders and 18,000,000 packages of rouge.

Students of diseases of the skin constantly emphasize the difficulty of determining the nature of any eruption. Red spots and slight inflammations may be due to disturbances of digestion, to special sensitivity to various food substances, to eczema, to any one of a half-dozen infections, and possibly to tuberculosis or to syphilis.

Since the tendency of some minor conditions is to recover without treatment, persons are likely to indulge in self-treatment and thus to neglect the more serious complaints to a time when a cure is far more difficult than when they are seen early. Cancer of the skin appears in its beginning as a small and rather unnoticeable spot; if seen early, it is easily controlled, but if stimulated by the use of all sorts of caustic acids or pastes it may become so serious as to produce death.

The craze for beauty has also resulted in the tremendous rise of the beauty parlor. It is easy to remember the time when our largest cities could boast of but five or ten hair-dressing establishments. Today in the Boroughs of Manhattan and the Bronx alone there are 1,177 hair-dressing establishments. Here women come not only for massages and permanent waves but also, frequently, for the treatment of diseases of the skin and for the removal of superfluous hair, moles, warts, or tumors.

These beauty parlors are the outgrowth, in many instances, of manufacturing establishments for the sale of cosmetics. Here cold cream masquerades under twenty different names and women purchase for considerable sums preparations which cannot possibly do the things that are claimed for them.

One finds “rose leaf cleansers,” “skin fresheners,” “face moulding creams,” “balsam tissue astringents,” “skin fatteners” and “skin thinners,” cleansing creams and vanishing creams, regardless of the fact that specialists in the disease of the skin have stated again and again that all that any cream can accomplish is simply to make the skin more pliable.

MAGIC OF PLASTIC SURGERY REPLACES SCARRING KNIFE

During the World War the mutilating injuries suffered by many of the men were assuaged through the development of new features in surgical technique.

Whereas formerly it was considered sufficient merely to sew up a wound, to amputate a shattered limb or to save life regardless of appearance, surgeons then began to consider operations with a view to the most presentable appearance possible after repair.

The technique itself is difficult but the war yielded a few masters in the medical service of each of the warring nations.

Stitching and Filling In.—In addition to making wounds more sightly by fine stitching, methods were evolved for filling in defects by transplanting tissues from points elsewhere in the body.

For instance, a wound under the eye was repaired by moving some of the loose tissue from the forehead or the cheek. This was left attached to its original blood supply until new vessels had come in and then the original attachment or pedicle was removed.

In one instance tissue taken from the upper part of the chest filled in the side of the face. Here mutilations were made sightly and at least a tolerable existence given to men whose lives otherwise might have been an agony of sensitiveness.

Transplanting Muscles.—In one case a man whose arm was lost at the shoulder had not even enough of a stump to attach an artificial limb.

By transplanting muscles and tendons from neighboring parts, surgeons were able to make a stump that served well for the attachment of an artificial limb with which the man was able to earn a livelihood.

The economic aspects of the situation are, of course, even more important than those relating wholly to the patient's appearance.

BUTTON IN SAVAGE'S SKULL FIRST PLASTIC SURGERY

Even the ancient savages used to make holes in the skull to let out the demons which they believed were responsible for disease.

Instances are recorded in which they also repaired such holes by the insertion of buttons of bone previously removed. In attempting to repair the ravages of disease, surgeons today sometimes fill in defects with substances of various kinds. At the recent meeting of the American Academy of Ophthalmology and Otolaryngology in Colorado Springs, one entire session was devoted to the newest points in this fine surgical art.

Removing an Eye.—When an eye must be removed because of inflammation or disease—and this sometimes done promptly to save the sight of the remaining eye—the cavity that is left may be extremely unsightly. Dr. Harold Gifford of Omaha told of various operations that are done to secure a good appearance. When an artificial eye is to be worn the socket must be made suitable to hold it. Sometimes paraffin is employed to develop a proper cavity, although most physicians fear this substance because its injection may produce irritations and overgrowth more unsightly and certainly more dangerous than the condition the paraffin was planned to correct.

Dr. Harry L. Pollock of Chicago told of several instances in which cancerous growths had followed the injection of paraffin by beauty doctors to relieve wrinkles or the sunken bridge of a nose injured by a blow. The appearance seemed beautiful immediately after the injection of the paraffin, but soon the irritation set in. Then the face became a mass of humpy tumors which had to be removed by surgery and which left serious scars.

Celluloid was tried for a while, but it also was discarded because the results were unsatisfactory.

Use of Ivory.—The newest and for the present apparently the best substance to use for repairing defects is ivory—either natural or synthetic. Dr. Pollock pointed out that this substance is more nearly like bone in its composition

than any other substance known. Both bone and cartilage are used also, but Dr. Pollock's experience inclined him to favor ivory over either.

Obviously this work is not for the amateur, but for the experienced worker. The so-called advertising beauty doctors seldom attempt it because the conditions under which most of them work are not conducive to successful surgery. They choose rather to delude their patients with promises of rapid and easy relief that are never fulfilled.

BEWARE OF BEAUTY DOCTOR WHO OFFERS GUARANTEES

Recently there came to medical attention the case of a woman who had been the victim of the search for beauty through surgery. She was the wife of a prominent merchant in a foreign country.

When she was eight years old she was operated on for the relief of tuberculous glands in her neck. She grew up, married successfully and gave birth to a handsome son.

But in the meanwhile her life, comfortable enough otherwise, was disturbed because the scars of her operation showed when she wore evening dress.

Tours Europe.—Finally she set out upon a tour of Europe. She arrived in Paris, where she consulted several eminent surgeons, all of whom advised against operation.

Then she heard, through the press notices of the theatrical papers, that American beauty doctors were doing marvels in such cases. In this country she reached finally a beauty doctor widely heralded through paid publicity secured by publicity agents.

She was intrigued and coaxed into an operation by cleverly written pamphlets detailing the success accomplished on worn-out actresses and movie stars. She decided at last not only on an operation on her neck but also on face lifting, face peeling, wrinkle removing and similar procedures.

No Anesthetic.—These operations were carried on in the office of the beauty doctor, without an anesthetic and rather crudely in the arrangements for cleanliness. A qualified surgeon works in a qualified and reputable hospital. Advertising beauty doctors are not admitted to reputable hospitals.

The woman finally left the ministrations of the beauty doctor. She had unsightly scars, worse even than those for which she had originally consulted him.

Her skin, irritated by the caustic chemicals used to peel it, continued to give off fine scales. The lifting operations, the loss of blood, the weeks necessary for recovery in a low-grade hospital in which the beauty doctor finally placed her, left the woman nervous, melancholy—mentally a wreck.

A reputable surgeon makes no guarantees of success in plastic surgery. The disreputable beauty doctor relies on the shame of the patient and her fear of ridicule to protect him when his surgery goes wrong, as it so often does.

EITHER HAIR OR HEALTH ENDANGERED BY DYES

Ever since the first woman looked into a polished surface and decided that her appearance would be greatly benefited by certain sophisticated modifications, the feminine sex has been tampering with its natural beauty.

Among the first of the features to receive attention has been the hair, long known as “woman’s crowning glory,” and now merely an added source of income to the tonsorial artist. The first hair dyes were made from vegetable substances. Then came metallic, chemical compounds, and more recently the synthetic dyes largely derived from coal tar products.

Among the vegetable dyes, henna is the best known example, but indigo wood extracts, sage and camomile have also been popular. Hair dyed with henna is light red, but one hour will make brown hair extremely red and give to white and blond hair a quite peculiar orange color, extremely artificial and unattractive.

The wood extracts and camomile produce all sorts of peculiar colors. They are reasonably safe, but suffer from the disadvantage that the dyes are difficult to prepare and the result not especially permanent. Most of these dyes do not penetrate the hair shaft, but merely coat it so that their repeated use tends to make the hair quite brittle.

Metallic dyes include combinations of lead, silver, copper, tin, and other metals. All metallic hair dyes, according to Dr. McCafferty, who has written extensively on the subject, must be considered as potentially dangerous when used for any length of time. The material may be absorbed into the body and the person so saturated with metallic poison that he develops serious symptoms.

Most of the dye substances offered on the market and advertised in publications are combinations of henna with metallic substances. The dye action depends principally on the pyrogallol and on the metal, with just enough henna present to permit the manufacturer to use the name “henna dye.”

The most common dye used today is the synthetic drug known as paraphenylene-diamine. Many persons are especially sensitive to this drug and develop severe eruptions following its use. In certain instances, eruptions on the back of the neck and on the arms follow the wearing of fur dyed with this substance. In Germany, the dye must always be marked with a "poison" label and most European countries have followed that example.

In this country, legislatures are only beginning to consider legislation of a similar character.

If a person is sensitive to paraphenylene-diamine, his scalp and face will begin to tingle and itch after its application, and this will be followed, in from 10 to 12 hours, by a swelling and blistering of the skin. The eyelids also may swell to such an extent that the person is unable to see.

Most of the cosmetic establishments use this dye for their work. The treatment given by most physicians consists in wet dressings of boric acid solutions and in small doses of X-ray to the region concerned. Following the subsidence of the preliminary symptoms, soothing creams are applied to the tissues. Dr. McCafferty points out that every substance used as a dye remover is a dangerous poison, which, if it does not injure the person using it, at least makes the hair extremely brittle. It is his belief that dye removers should be legislated out of the beauty shops.

SKIN ERUPTIONS LAID TO POISON IN DYED FUR

Every winter physicians who specialize in diseases of the skin are asked to see cases of eruptions around the throat and wrists resulting from poisoning with incompletely or improperly dyed furs.

The principal symptoms are blotches and swelling with itching and burning, so severe in some cases that the person cannot sleep at night.

Rabbit Becomes Beaver.—The trouble is found in most instances to result from rabbit that is dyed to imitate beaver, although all sorts of furs may be involved.

In these cases the poisonous substance is usually a compound of paraphenylene-diamine, the same poisonous substance that causes irritation of the scalps of those who attempt to dye gray hair black.

Many foreign nations have introduced laws to prevent the use of this substance in hair dyes and to regulate the fur-dyeing industry. Similar laws have been passed in some of our states, but there is as yet no nation-wide regulation regarding the use of this substance.

Investigate Cases.—Not all persons are equally sensitive to this dye. Some may wear furs dyed in this manner without symptoms, but those who are sensitive develop severe reactions.

Several investigative committees are attempting to secure a record of all the cases of this type that occur.

If anyone knows of such an instance, the report may be sent to the Committee on Dye Poisoning of the American Medical Association, 535 N. Dearborn Street, Chicago.

MOST MOLES HARMLESS, BUT WATCH BLACK ONES!

It has been estimated that there are five moles on an average on every person in the city of Chicago.

If all of these were to be removed, it would require more than 15,000,000 separate surgical operations.

The history of moles would be an extremely interesting topic, since they were long regarded as of great significance—sometimes considered the work of evil spirits; sometimes thought to be due to the influence of the moon or the stars, and occasionally thought to be of importance in determining configuration of the body.

Superstition.—Charts of moles were prepared so that one might tell by observation of those that were visible where to expect others that might be invisible. Needless to say, all of this was superstitious and without the slightest scientific importance.

As long as a mole does not disfigure the appearance, it is best disregarded, unless under certain circumstances it becomes irritated and subject to the unusually rapid growth that is characteristic of cancer.

If a mole is extremely brown or black, it has in it an unusual amount of a pigment called melanin, which is responsible for giving the color to the hair, the eyes, and the deeper layers of skin.

Somehow, the presence of this substance seems to be associated with dangerous possibilities, so that a severe injury or irritation of a deeply pigmented mole may cause it to suddenly develop rapid growth.

Watch Black Ones.—The most dangerous moles are those that are jet black. If these are situated where they are constantly rubbed or irritated by collars, garters, shoes or other garments, they should be most carefully watched.

The danger signs have been listed by some doctors as: (1) a gradual increase in the size of the mole; (2) deepening or spreading of the color; (3)

ulceration or infection; (4) pain or soreness in the mole.

If any of these symptoms appear, the person concerned should promptly consult a physician. If they do not appear, the mole should be left alone and not teased by irritation, rubbing, picking, scratching, or half-hearted attempts at its removal.

FLOWERS CARRY POISON

Some persons are especially sensitive to contact with toxic substances derived from plants. The poison ivy, oak, and sumac may cause severe eruptions of the skin in persons susceptible.

When mah-jong first became popular, many persons had eruptions of the fingers and of the skin of the face from contact with the lacquer on the mah-jong boxes, due to a special sensitivity that they possessed to a poison in the lacquer which it was discovered had been made by utilizing the juices of certain Japanese plants.

Handling of Bulbs.—Now a British physician has discovered eruptions on the hands of some persons from the handling of flower bulbs, and has given the name “lily rash” to this type of disturbance.

It followed the cutting of the stems of the flowers, chiefly the narcissus, and from handling bulbs of the hyacinth, daffodil, narcissus and tulip.

An investigation was made in several establishments devoted to the sale of bulbs, and a small proportion of packers and sorters of bulbs were found to be suffering from an eruption extending under the nail, where splitting of the skin caused considerable pain.

The longer the nails were worn, the more severe was the condition. Moreover, the inflammation of the skin, which occurred after a few days' work in handling the bulbs, was progressive until the worker began to use gloves.

All the workers were inclined to blame the tulip bulbs. Some of the observers thought that the eruptions were due to friction from the rough sides of the bulbs, and were ready to place the responsibility on the hyacinth, narcissus and daffodil.

An investigation showed, however, that the tulip bulb is surrounded with a single layered covering which spreads easily and that it holds a somewhat acrid juice. When this juice was applied to the skin underneath the finger

nail it promptly produced irritation, even without any breaking of the skin due to rubbing.

The obvious suggestion is that persons who are likely to handle tulip bulbs in any considerable numbers ought to wear protective gloves.

SNOW OR COLD WATER BEST IN TREATING FROSTBITE

When a portion of the body not properly protected is exposed to intense or extreme cold, the tissues become affected. If the cold is sufficiently intense and the exposure is sufficiently prolonged, the part becomes frozen.

When the circulation of the blood is sluggish, as in the very young, the old and the sick, suffering from cold is likely to be more severe.

In the same way those parts of the body in which the circulation is least active, and which are least protected by clothing, as the ears, the hands and the feet, are most likely to be affected.

Blisters May Form.—In mild frostbite there usually is merely tingling and slight pain. If the cold is more intense and the exposure more prolonged, blisters form with serum or blood inside of them.

If the freezing is still further prolonged, the entire part becomes gangrenous because of the congealing of the blood within. Then the tissues appear bluish, shrunken or wrinkled and are without sensation or the power of motion.

The best treatment for frostbite at first is friction with snow or cold water in a cold room, the changes to a warmer atmosphere being brought about gradually.

After the friction the feet or hands should be swathed in cotton-wool held in position by loose bandages. If there are blisters or any discolorations of the tissues, a physician should be seen promptly.

On the speed with which proper treatment is given may depend the saving or loss of a limb.

Chilblains.—Chilblains, which also are associated with a sluggish circulation, usually produce burning heat with itching and redness, and are likely to follow prolonged exposure to cold combined with dampness.

They may be prevented by wearing warm, loose woolen stockings and warm shoes. The feet should be bathed in warm water daily, and after the bath should be rubbed briskly, quickly dried and dusted with a plain dusting powder.

WASH YOUR NECK WELL TO WARD OFF BOILS

Those frequent and unwelcome visitors—boils—always are due to skin infection. The greatest preventive is constant cleanliness of the skin, particularly of the back of the neck or other parts commonly affected.

In men the back of the neck is the part most commonly affected, usually because of the constant rubbing of the collar on the hair of the lower part of the back of the head.

May Be Serious Signal.—Certain diseases, such as diabetes, seem to predispose the person to infection with boils, and the constant recurrence of boils should be the signal for a visit to the doctor.

The person who succumbs readily to skin infections lacks bodily resistance. This resistance of the skin is associated with the presence in the blood of substances that will overcome bacteria.

The constantly recurring warning to keep the body in the best physical state by proper diet, proper cleaning and prompt attention to infections in the ear, nose and throat is here of the greatest importance.

When a boil occurs the parts around it are usually shaved, since the infection spreads from one hair follicle to another. All sorts of antiseptics may be applied and fail to control the boil unless the tissues of the body wall it off and cause it to “come to a head.” Heat aids this process by bringing a good supply of blood to the part affected.

Surgical Attention.—A boil should not be opened by some friend who happens to think himself a surgeon merely because he does not faint at the sight of blood. Improper opening or drainage of a boil may be exceedingly serious by spreading the infection to the body generally.

The surgeon will open the boil with proper precautions against extension of the infection and will see to it that the boil drains properly.

The germs that cause boils are small round organisms, commonly seen in clusters when stained and studied with a microscope. Some physicians build up the resistance of the chronic sufferer from boils by preparing a vaccine from the patient's own bacteria.

PSORIASIS ONE OF THE MOST ANNOYING SKIN FAULTS

Of all of the diseases of the skin that affect the human body, one of the most annoying is called psoriasis. In this disease, the skin becomes inflamed and there develop round patches which are red at their base and which are covered by dry, white scales.

These patches slowly enlarge, never developing into blisters, but continuing to give off scales. The disease usually begins on the back of the arms and the front of the legs, just below the elbows and knees, and may involve also the scalp and the lower part of the back, but seldom appears on the face.

The disease continues for months and years, sometimes better, sometimes worse. The patient is seldom disturbed in general by the condition, except by its annoying appearance and the development of the scales.

A disease of this type has been treated by all of the methods known to medical science, and the fact that so many methods have been suggested is an indication that none has proved especially satisfactory. Any good specialist in diseases of the skin is able to clear up the lesions temporarily by the use of proper drugs, and usually recommends a change in the diet involving reduction of protein substances and of meat.

In a recent consideration by the leading specialists in diseases of the skin of Great Britain, it was pointed out that the best results today are secured by local treatment, with attention to the general hygiene of the body, and that the value of the various special forms of treatment mentioned in medical literature have not yet been established.

A person who suffers with this disease will do well to select a specialist in diseases of the skin and to follow the general routine laid down.

The chronic character of the disease and its apparent incurability cause persons affected to travel from one specialist to another, and to consult all sorts of fads and quackery in their search for relief. Until some specific

curative method is discovered, they will in this way merely waste funds which might better be conserved for useful treatment.

“CAULIFLOWER EARS”

Among the first of the beauty operations to attract public attention was the correction of what the pugilists call a cauliflower or “tin” ear. This term is applied to the result of a powerful force sharply administered to the appendages on the sides of the prizefighter’s visage. Immediately thereafter the blood vessels beneath the skin burst and the blood pours out into the membranes which cover the cartilage or gristle of the ear. The blood distends the tissues of the ear to the extent of the blood pressure and until the blood coagulates. The swelling thus formed may gradually be absorbed and the ear return to its normal state, but in many instances tissue forms out of the mass and when this contracts the ear is distorted into the cauliflower shape. Dr. Samuel Iglaur has pointed out that it is the left ear that is usually of the cauliflower variety, since this ear receives most of the wallops from the right fist of the opponent. Cauliflower ears are also seen frequently in wrestlers, and before the development of headgear were not unusual in football players.

After the pugilist develops affluence and is ready to retire to the stage or the screen he is usually ready to submit himself to remodeling and decorative surgical art. If he develops this vanity previous to his retirement, the revamped ear is likely to become a special target for the opposition the moment he steps into the ring.

FLOPPING EAR NEEDS CAREFUL OPERATION

Next to the cauliflower ear, no other deformity of this particular organ is so frequently the subject of correction as the outstanding ear. This is usually a congenital development and one which is likely to give much distress, particularly to a girl who is otherwise handsome. Many persons have thought that outstanding ears could be corrected merely by sewing the ear to the skin back of it. However, in practice it has been found that the skin will stretch promptly and the ear sag into a new and perhaps more undesirable position. The operation is, therefore, most intricate, involving actual transplantation of part of the cartilage of the ear to the bony covering of the skull behind the ear.

Such decorative surgery is comparatively recent in medicine. Perhaps the first operation for correction of outstanding ears was devised in 1861. Nowadays, with the increasing prosperity, publicity and vanity, such operations have become fairly frequent.

The reliable cosmetic and plastic surgeon is usually associated with a hospital that has been classified by the American Medical Association and the American College of Surgeons as a reputable institution. His work is done under the best surgical conditions, with the assistance of well trained nurses and with all of the cleanliness that is absolutely necessary if surgery of any type is to be successful.

It has been said that the persons who seek cosmetic surgery are likely to be dissatisfied with any result short of perfection, and that they submit to operation after operation, falling more and more into the net of the unprincipled surgeon, if they happen to have begun their quest for beauty with an advertising quack.

The great campaign of education about quackery carried on for many years has resulted in the development of new methods of advertising by the unprincipled surgical impresario. He is far too shrewd to indulge himself in purchased advertising space in newspapers or periodicals. He, therefore,

secures a publicity agent who is as likely as not to be a reporter on some local newspaper, willing to eke out a narrow income by playing both ends against the middle. This enterprising journalist takes payment from the beauty surgeon for securing space for him in the daily press, and submits to his newspaper news items regarding weird operations performed by the beauty surgeon on actors, pugilists and other notorious persons. The result is to bring a flood of less notable but equally simple kitchen mechanics, stenographers, elderly housewives and other shallow-minded searchers for beauty into the net.

RUBBER “REDUCERS” ARE FOES OF COMFORT BUT NOT OF FAT

In the craze for reduction which agitated most of the women of the United States during the past few years, many of them undertook all sorts of exercises, diets and the use of all kinds of apparatus to remove what they considered superfluous weight.

Among the most widely advertised articles were corsets and brassières, made of pure gum rubber, which were supposed to be worn next to the skin. It was perhaps the notion of those who promoted these devices that they would squeeze the flesh into a more solid form, perhaps causing the body to eliminate the superfluous matter from inside.

There is not, of course, the slightest scientific reason to believe that such apparatus could do anything of the kind. The chief effect of wearing rubber garments next to the skin is that they prevent perspiration from evaporating. Since the perspiration is rubbed into the skin, it is likely to produce irritation.

Some persons have severe irritation of the skin from contact with the rubber itself. The rubber garment does not increase the sweat, but simply causes it to remain on the surface. Certainly it does not cleanse the skin, but rather tends to make it accumulate deposits of waste products.

The sweat regulates the temperature of the body by evaporation from the surface. It also keeps the skin soft by keeping it moist and well lubricated. Rubber garments prevent evaporation of the sweat, which produces laceration of the skin.

Certainly they cannot in any way aid the lubrication of the skin, since retention of salts and other substances is likely to make it drier and to cover it with crusts.

BE CAREFUL IN USE OF LAXATIVE GUMS

It is safe to say that more proprietary medicines of a laxative character are sold in this country than in any other.

The vogue of chewing gum is also particularly an American institution. It is not surprising therefore that a manufacturer has begun the sale and distribution of chewing gum into which is incorporated a quantity of a well known laxative drug.

Caution in Usage.—While chewing gums thus medicated may be safe as laxatives, there are excellent reasons why this drug should not be sold indiscriminately. Samples were distributed in the streets of some cities, until boards of health prevented such distribution because the samples fell into the hands of children and there were serious results.

Besides, some persons are especially sensitive, and react with severe inflammation of the skin when they take even a small dose of this remedy.

The second use to which chewing gum has been put has been to incorporate drugs alleged to have special properties for reducing obesity.

Thyroid Extract.—Anti-fat chewing gums contain thyroid extract, which is dangerous; drugs that irritate the stomach and disturb the appetite, which are also dangerous, and drugs which have no power whatever, which is foolish. The United States government has recently issued fraud orders against several chewing gums of this character.

The person who uses the gum is told that in addition to taking the remedy she should walk five miles a day, go through certain exercises, and cut down the diet. These practices will bring about a reduction in weight just as well without chewing the gum.

OVEREATING HARMFUL AFTER MIDDLE LIFE

Most of the illnesses that afflict the man beyond middle age are due to the fact that he does not realize his intestinal limitations.

The most pernicious misdemeanor after middle life is overeating. The person who works out-of-doors is, of course, able to dispose of more and heavier food than does the brain-worker.

The brain-worker who tries to eat meat three times a day will find himself, in most instances, in better condition if he eats meat only once each day, but in this condition quantity of food is more important than its nature.

Cooked Foods.—The human being gradually has been adapting himself to more and more refined and thoroughly cooked foods. Nevertheless, it is not desirable for him suddenly to change to foods that are coarse and indigestible.

These perhaps will aid elimination, but at the same time they will irritate the delicate lining of the intestines and permit the more easy entrance of bacteria into the body.

Too much starch, protein or fat is harmful because it will not be digested fully and will encourage the growth of bacteria in the intestines. As one becomes older his tolerance for sugars becomes less, and if he eats too much sweet or starchy foods, he is likely to develop disorders of sugar elimination.

Insurance Records.—All health authorities are agreed that the greatest danger for the man beyond middle age is overeating. The records of great insurance companies show that the life expectancy of the man slightly underweight at this age is greater by far than that of the man who is overweight.

As the old farmer expressed the matter, pigs would live longer if they did not make hogs of themselves.

Transcriber's note

Minor punctuation errors have been changed without notice.

Spelling was retained as in the original except for the following changes:

Page 3 : “Hademan-Julius Company”	“Haldeman-Julius Company”
Page 3 : “UNITED STATES OF AMER”	“UNITED STATES OF AMERICA”
Page 19 : “with dangerous possibilities”	“with dangerous possibilities”
Page 32 : “proteins or fats is”	“protein or fat is”
Page 32 : “sweets or starchy foods”	“sweet or starchy foods”

*** END OF THE PROJECT GUTENBERG EBOOK THE CARE OF
THE SKIN AND HAIR ***

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